MENTALHEALTH ASSISTANT

Detailed RAG Knowledge Base

A comprehensive, structured reference based on the uploaded WHO, UNICEF and ILO documents

Purpose: Support retrieval-grounded answers about everyday stress, self-help, students and young people, perinatal mental health, work-related psychosocial risks, support-seeking, and safe chatbot boundaries.

Source boundary: This is a newly written synthesis, not an official publication of WHO, UNICEF or ILO. Keep and index the original PDFs as the primary evidence sources. Do not use this document as a clinical guideline or diagnostic tool.

Expanded edition | 17 October 2026

How to use this knowledge base

This edition is deliberately detailed rather than a short overview. It organizes broad concepts into retrieval-friendly sections, gives examples of supportive chatbot wording, distinguishes different audiences, and specifies how the application should use evidence and citations. It is intended to supplement the original uploaded PDFs, not replace them.

Source hierarchy

1. Use the most relevant passage from an original official source PDF whenever one is available.
2. Use this synthesis to provide plain-language framing or to connect related themes, while retaining a citation to the original source when that source supports the answer.
3. If the original source and this summary appear inconsistent, prefer the original source and flag the inconsistency for review rather than silently reconciling it.
4. If no approved source provides enough evidence, say so. Do not fill the gap using ungrounded model knowledge or external websites if the application policy is document-only.
5. Do not treat similarity scores alone as proof that a passage supports a claim.

Approved source set

Source document	Main use in retrieval
WHO, Doing What Matters in Times of Stress: An Illustrated Guide (2020)	Grounding, unhooking from difficult thoughts, acting on values, making room, engaging, being kind.
WHO, Psychological Self-Help Interventions (2026)	Structured low-intensity self-help concepts and the approaches named in the manual; use the original for protocol detail.
WHO, Guidelines on Mental Health at Work (2022)	Organizational interventions, manager and worker training, individual interventions, return to work and supported employment.
ILO, The Psychosocial Working Environment: Global Developments and Pathways for Action (2026)	Psychosocial risks, work environment, prevention and policy pathways.
WHO and ILO, Mental Health at Work: Policy Brief (2022)	Prevent, protect, promote and support approaches; roles of employers, workers and policy-makers.
WHO and UNICEF, Mental Health of Children and Young People: Service Guidance (2024)	Public mental health, service delivery, access barriers and systems for children and young people.
WHO, Guidelines on Mental Health Promotive and Preventive Interventions for Adolescents (2020)	Adolescent-focused promotion and prevention; not an individual diagnostic manual.
WHO, Guide for Integration of Perinatal Mental Health in Maternal and Child Health Services (2022)	Perinatal mental health in maternal and child health services.
stress Management.pdf	Stress-management content; verify author, date, publisher and exact scope before prioritizing it.

Important limitation

The uploaded documents have different purposes and intended audiences. A service-planning guide is not necessarily a self-help protocol; a workplace guideline is not a legal ruling; adolescent guidance should not automatically be applied to adults; and perinatal guidance should not be generalized to everyone. The chatbot must preserve these distinctions.

1. Mental well-being and supportive communication

The source collection addresses well-being through coping skills, prevention, service access, and the conditions in which people live, study and work. A useful chatbot response should be respectful, practical and proportionate to what the user has shared. It should not assume that a short message reveals the person's full circumstances.

A reliable conversational pattern

1. **Acknowledge:** Reflect the user's concern in neutral, non-judgmental language.
2. **Clarify:** Ask one focused question only if the answer depends on information that is missing.
3. **Offer choice:** Ask whether the person wants listening, general information, or help considering a practical next step.
4. **Keep it manageable:** Offer one or two realistic options rather than an overwhelming list.
5. **Respect limits:** Do not diagnose, prescribe, promise an outcome, or imply that the chatbot knows the person's situation better than they do.
6. **Follow up appropriately:** If the problem is persistent, seriously affects daily life, or involves safety, encourage suitable human or professional support.

Examples of helpful wording

> *“That sounds difficult. Would it help to talk through what is happening, or would you prefer to choose one small next step?”*
>
> *“I can share general information from the documents available to me. I cannot determine a diagnosis from a chat, but I can help you find information about support options.”*

Communication to avoid

- “Just be positive,” “Other people have it worse,” or any response that dismisses the person's experience.
- Claims that one exercise or habit works for everyone.
- Assumptions based on gender, age, job, family role, culture or student status.
- Overconfident labels such as declaring that a user has a disorder based on a short message.
- Promises of absolute confidentiality or claims that the assistant can keep someone safe by itself.

2. Understanding stress

Stress may arise in response to demands, uncertainty, conflict, difficult experiences or a sense that available resources are insufficient. The user's own description should guide the conversation. Stress is not itself a diagnosis, and similar experiences can have different causes and effects in different people.

Questions the chatbot may help a user consider

- What is the most pressing part of the situation right now?
- Is there one part of the problem that can be acted on today?
- Is the user looking for emotional support, practical planning, or information about professional help?
- Is the situation affecting sleep, work, study, relationships or everyday functioning?
- Is there an immediate safety concern that requires the safety pathway rather than ordinary advice?

A practical stress response

The WHO illustrated stress guide emphasizes grounding, unhooking from difficult thoughts, acting on values, making room for feelings, engaging, and being kind. These concepts can be summarized in plain language, but detailed exercises should be retrieved from the original guide so that their intended instructions are preserved.

1. Pause long enough to notice the present situation.
2. Name the immediate difficulty without adding a judgment about personal worth.
3. Choose an attention exercise or a simple practical action, if the user wants one.
4. Identify one value or priority relevant to the situation.
5. Take a small step consistent with that value.
6. Review whether the step was useful; if not, consider another safe option or human support.

3. Grounding, attention and mindfulness

Grounding practices invite attention back to the current environment or activity. The WHO stress guide includes grounding as one of its core skill areas. A chatbot should offer these exercises as optional practices rather than treatment, and should not suggest that they guarantee relief.

Sensory grounding

A simple sensory exercise can invite the user to notice details in the environment—for example, objects they can see, sounds they can hear, or the feeling of their feet touching the floor. The user may adapt the exercise to their circumstances. Avoid forcing a particular sensory sequence when it is inaccessible, uncomfortable or inappropriate.

Grounding through an ordinary activity

Invite the user to describe one object, notice the steps in making a cup of tea, or attend to a simple task. The purpose is to practise directing attention, not to suppress or deny the problem. Afterward, help the user choose what they want to do next.

Comfortable breathing pause

If the user requests a breathing exercise, keep it gentle and optional. Invite comfortable, natural breathing rather than forceful deep breaths or prolonged breath-holding. If the user feels dizzy, short of breath, or more distressed, tell them to stop the exercise and return to normal breathing. Do not use a breathing exercise instead of urgent help when safety is at issue.

Unhooking from difficult thoughts

A thought may feel convincing without being a confirmed fact. For example, “I will definitely fail” expresses a fear about a future outcome. A supportive response can help separate what is known from what is feared, while still acknowledging that the concern feels real. The assistant should not argue harshly, invalidate the user, or guarantee success.

Acting on values

Values are qualities or directions that matter to a person, such as learning, care, safety, honesty, connection or responsibility. The chatbot may ask what matters most in the situation and help identify one small action aligned with that value. This approach does not mean a person must tolerate unsafe circumstances or solve structural problems through attitude alone.

Making room and kindness

Making room means allowing an uncomfortable feeling to be present without automatically letting it determine every action. Kindness includes reducing self-blame, recognizing limits, asking for support and responding respectfully to others. These are optional skills, not moral requirements; a user should never be blamed for continuing to feel distressed.

4. Students, exams and academic pressure

Students may describe exam anxiety, assignment overload, procrastination, fear of failure, family expectations, peer comparison, college transitions, financial concerns or uncertainty about the future. The chatbot should ask which issue matters most rather than assume the cause from the user's student status.

Supportive steps for academic overload

1. Identify the nearest deadline or the task causing the most pressure.
2. Break the task into a small action that can be started now.
3. Separate urgent tasks from important but less urgent tasks.
4. Set a realistic work period and a stopping point; include breaks where possible.
5. Ask a teacher, classmate, adviser or counsellor for specific help when useful and safe.
6. Treat a disappointing result as information for future planning, not a complete measure of a person's worth.

When a student fears failure

Acknowledge the fear without promising that the outcome will be positive. Ask what remains to be done and how much time is available. Help the student choose one useful next action, such as reviewing a topic, attempting a practice question, or asking for clarification. Do not claim that a particular schedule guarantees success.

Procrastination and task initiation

If the student feels unable to begin, suggest reducing the first step until it is specific and manageable—for example, opening the document, writing three headings, or studying one concept. The aim is to make starting easier, not to shame the student. If the difficulty is persistent or affects broader functioning, encourage appropriate support.

Loneliness, bullying and peer conflict

Listen without blaming the student. Ask what kind of support they want and whether there is a safe person they can approach. Avoid advising retaliation or risky confrontation. If there is a threat, abuse or immediate danger, prioritize real-world safety and a trusted, appropriate adult or service.

5. Children, adolescents and young people

The WHO and UNICEF service guidance addresses public mental health, service delivery, systems and barriers to care for children and young people. The WHO adolescent guidance focuses on promotive and preventive interventions. These documents should be used within their intended age and service context; they do not authorize the chatbot to diagnose or treat a child independently.

Age-aware communication

- Use clear, simple language and avoid clinical labels unless explaining a term found in the source.
- Do not request unnecessary personal information from a child or young person.
- Recognize that a child may have limited control over family, school or living conditions.
- Encourage support from a safe, trusted adult and qualified service when appropriate.
- Do not automatically direct a child to an adult who may be unsafe or causing harm.
- Do not promise secrecy when there is a serious safety concern.

Helping a young person talk about a problem

A helpful opening may be: “You do not have to explain everything at once. What is the hardest part for you right now?” The chatbot can offer choices such as talking to a trusted adult, writing down a concern, or finding an appropriate service. It should avoid pressuring the young person to disclose more than they want to share.

Prevention and promotion are not diagnosis

Promotive and preventive guidance is intended to support well-being and reduce risks at relevant individual, family, school, community or service levels. It should not be converted into a checklist that declares whether a young person has a mental disorder. For specific recommendations, retrieve the original WHO adolescent guidance and preserve the context in which the recommendation is made.

Service access and barriers

The WHO/UNICEF service guidance covers barriers and service systems. A chatbot may acknowledge that access can be affected by availability, cost, stigma, distance, trust or uncertainty about where to begin, but should cite the source passage that supports a specific statement. It should not invent local service availability or eligibility rules.

6. Perinatal and maternal mental health

The WHO guide on integrating perinatal mental health into maternal and child health services focuses on service integration and the period around pregnancy and childbirth. The chatbot can provide general information from this source and encourage a person to discuss persistent or concerning emotional changes with a qualified health professional or maternal and child health service.

Supportive response principles

- Do not assume a parent feels only happiness or that distress reflects a personal failure.
- Acknowledge the experience without diagnosing the cause.
- Encourage appropriate professional support for persistent or concerning changes.
- Offer practical help-seeking steps, such as writing questions for an appointment or identifying a trusted support person.
- Do not provide individualized medication, pregnancy or treatment instructions.
- Treat immediate safety concerns as urgent and use the application's separate safety pathway.

Keep scope specific

Perinatal mental health guidance should not be generalized to every woman, every parent, or every mental-health question. For specific service recommendations, eligibility criteria or clinical procedures, retrieve the original guide and preserve its intended audience and setting.

7. Everyday routines and self-care

The uploaded self-help materials support practical coping and structured self-help approaches, but the chatbot should not present a personalized health plan unless an approved source supports it. General, low-risk suggestions may include choosing a manageable routine, taking breaks, staying connected to supportive people, and seeking advice when persistent changes cause concern.

- Choose one realistic adjustment rather than trying to change everything at once.
- Set a clear, manageable start and stop point for a task.
- Include rest or enjoyable activity where circumstances allow.
- Reach out to a supportive person instead of relying only on a chatbot.
- Seek qualified advice if changes in sleep, mood, appetite, energy or daily functioning persist or cause concern.

8. Workplace mental health: understanding the environment

The WHO guidelines, WHO/ILO policy brief and ILO global report treat workplace mental health as a shared concern involving working conditions, organizational systems and individual support. Work can provide routine, relationships, income, purpose and achievement; unsafe or poorly managed work conditions can also undermine well-being. A chatbot should not frame all work-related distress as an individual failure to cope.

Examples of psychosocial risks

- Excessive workload, unrealistic deadlines or sustained long hours.
- Unclear responsibilities, low control over tasks or insufficient resources.
- Job insecurity, inadequate or unpredictable pay, or precarious employment.
- Poor social support, workplace conflict, harassment, discrimination or unsafe conditions.
- Difficulty balancing work and personal responsibilities.
- Work cultures that stigmatize mental-health concerns or discourage people from seeking support.

Individual experience and organizational responsibility

Coping skills may help some workers manage difficult moments, but they cannot substitute for action on harmful working conditions. If a user describes an unreasonable workload, bullying or unsafe work, the assistant should recognize both the person's experience and the possible organizational dimension. It should not imply that breathing exercises or resilience training alone resolve structural risks.

A worker's practical options

1. Identify the specific pressure, such as workload, unclear priorities, conflict or job uncertainty.
2. Consider whether a practical change or clarification is possible.
3. Where safe and feasible, discuss priorities, resources or deadlines with an appropriate person.
4. Consider available support channels such as a trusted colleague, worker representative, occupational health service, employee assistance programme or qualified professional.
5. Use jurisdiction-specific legal or policy advice only when an approved source for that jurisdiction is available.

Manager and employer responsibilities

- Make roles, responsibilities and priorities clear.
- Review workload, staffing, resources and avoidable psychosocial hazards.
- Promote respectful communication and routes for raising concerns without retaliation.
- Provide appropriate manager and worker training, while recognizing that training alone cannot correct harmful conditions.
- Support fair participation, reasonable accommodations and return-to-work planning where appropriate and consistent with relevant policy.
- Respect privacy and avoid pressuring workers to disclose diagnoses or unnecessary personal information.

Unemployment and job transitions

Unemployment, job searching and career transitions can involve uncertainty, rejection, financial pressure and changes to routine. A supportive chatbot may help a user choose a manageable job-search action, maintain social connection and identify relevant employment or community support. It should not blame the user or imply that attitude alone determines employment outcomes.

9. Work-related scenarios and response patterns

Scenario: “I have too much work and cannot finish.”

Acknowledge the pressure. Ask which deadline or task is most urgent. If appropriate, help the user prepare a concise request to clarify priorities or discuss a realistic deadline. If the user describes persistent overload, identify it as a possible work-design issue and cite relevant WHO/ILO material when retrieved. Do not assert that a manager must take a specific legal action unless an applicable source supports it.

Scenario: “I am a manager and my team seems exhausted.”

Suggest reviewing workload, staffing, clarity of expectations, control over tasks, support and the channels through which staff can raise concerns. Encourage respectful, non-punitive conversations and organizational review rather than asking employees to disclose private diagnoses. Ground specific recommendations in the WHO workplace guideline or policy brief.

Scenario: “I am returning to work after a mental-health-related absence.”

Respond respectfully and avoid assuming what the person can or cannot do. The WHO workplace guideline includes material on returning to work after absence associated with mental-health conditions. Retrieve the original recommendation before describing a process; do not invent a universal return-to-work plan or disclose private information to an employer.

Scenario: “Can my employer dismiss me?”

Do not provide a definitive legal conclusion without a relevant, current source for the user's jurisdiction. Explain that workplace rules differ and suggest seeking appropriate local legal, worker-representative or professional advice. The uploaded WHO/ILO sources provide general mental-health-at-work guidance, not a substitute for jurisdiction-specific legal advice.

10. Family, friends and caregivers

Supportive relationships can be an important source of help, but supporters do not have to solve every problem. A chatbot can help users listen respectfully, ask what kind of support is wanted and offer specific assistance when welcomed.

Helpful ways to respond

- “Would you like me to listen, or would you prefer help thinking through options?”
- “What feels most difficult about this today?”
- “Is there someone you trust who could support you with this?”
- “We can focus on one manageable next step rather than solving everything at once.”

Caregiver boundaries

Caregivers may need support themselves. Encourage realistic expectations and shared responsibilities where possible. Do not imply that family support replaces qualified care when care is needed. Keep dignity, privacy, consent and safety central.

11. Seeking professional support

Self-help is not suitable as the only response in every situation. Encourage appropriate professional support when distress persists, intensifies, affects daily functioning, or raises safety concerns. The WHO psychological self-help manual has a defined scope and specific approaches; retrieve the original manual for detailed procedures and eligibility conditions rather than inferring them from this summary.

A neutral way to explain the chatbot's limits

> *“I can share general information from the documents available to me, but I cannot diagnose you or replace a qualified professional. If this is affecting your daily life or you are worried about safety, consider contacting an appropriate health professional or local support service.”*

The assistant should not

- Diagnose a condition based on a conversation.
- Recommend, start, stop or change medication.
- Claim that a self-help exercise is a guaranteed treatment.
- Tell a user that professional help is unnecessary.
- Use a screening result as a diagnosis.
- Present service availability, eligibility or treatment procedures unless supported by an approved source.

12. Crisis and immediate safety

This knowledge base is not an emergency service and is not a complete, real-time crisis directory. The application needs a separate, tested safety pathway. It must not wait for a normal vector search to succeed before responding to a possible immediate danger.

Operational safety principles

- If a user indicates immediate danger or says they may harm themselves or another person, prioritize safety and human support over ordinary wellness information.
- Respond calmly and directly; do not shame, argue, or give a long generic lecture.
- Encourage contacting local emergency services or a trusted person who can be physically present, as appropriate to the situation.
- Do not rely only on a mindfulness exercise, retrieved passage or LLM classifier.
- Do not promise that the chatbot can keep the user safe.
- Use verified, current contact details; record the verification date and responsible reviewer.
- Test the safety pathway when the vector database is empty, unavailable, or returns irrelevant results.

India-focused deployment

Before deployment in India, verify current official mental-health support and emergency details from authoritative government sources. Confirm operating hours, language support, eligibility and service scope. Store reviewed contacts separately from the ordinary PDF index and keep a last-verified date. The uploaded documents alone should not be treated as proof that a number or service remains current.

13. RAG answer rules and abstention

The application goal is document-grounded answering: ordinary factual responses should be supported by the approved local corpus. A prompt alone is not enough to guarantee this. The backend should pass only retrieved evidence as the answer context, check relevance, validate citations and test abstention.

Recommended answer policy

- Answer only the portion of a question supported by retrieved passages.
- Separate a source-supported statement from a suggestion or interpretation.
- Use the source's intended audience and context.
- Do not add unsupported statistics, clinical claims, treatment instructions or legal conclusions.
- Say explicitly when the corpus does not contain enough information.
- Use a clarification question when the user's request is ambiguous and the missing detail affects the answer.
- Use original source citations rather than citing this synthesis alone whenever the original source is available.

Example insufficient-evidence response

> *“I could not find enough information in the approved documents to answer that reliably. I do not want to guess. If you can narrow the question, I can check the available sources again.”*

Evidence quality checks

1. Does the retrieved passage answer the actual question, rather than merely share similar keywords?
2. Does the passage support each important factual claim in the draft response?
3. Is the source suitable for this user's context and age group?
4. Is the source current enough for the type of claim being made?
5. Can every displayed citation be mapped to a real source file and page?
6. If one check fails, narrow the answer, retrieve again, ask a clarification question or abstain.

14. Chunking, metadata and citations

RAG quality depends on preserving context during ingestion. Each text chunk should retain its origin so the assistant can show users where an answer came from and so developers can audit incorrect answers.

Recommended metadata fields

Field	Why it matters
source_id	Stable internal identifier for the source document.
source_filename	Exact PDF filename as stored in the approved source folder.
document_title	Readable title extracted from the publication.
publisher	WHO, UNICEF, ILO or verified author/publisher where known.
publication_year	Helps distinguish editions and identify older guidance.
page_number	Supports user-visible citations and auditability.
section_heading	Preserves the source's organization and topic context.
source_type	For example: official guideline, policy brief, service guidance, illustrated guide, or project synthesis.
ingestion_version	Makes it possible to identify which index build produced a chunk.

Ingestion workflow

1. Keep original PDFs unchanged in the approved documents directory.
2. Extract text by page where possible. Inspect scanned or image-heavy pages when text extraction is incomplete.
3. Split by headings and paragraphs where feasible; use overlap only to preserve continuity.
4. Do not combine unrelated topics into one chunk simply to reach a target size.
5. Attach source metadata to every chunk before embedding.
6. Store embeddings and text in persistent ChromaDB.
7. Rebuild or update the index when source files change, and avoid duplicate chunks.
8. Run test questions and verify that the citation points to the correct original page.

Citation format

> *[WHO guidelines on mental health at work.pdf, p. 25, section: Recommendations for training managers]*

This is an example format only. The page number and section must come from actual metadata; do not hard-code or guess citations. Be consistent about whether page numbers refer to the PDF viewer page or the publication's printed page.

15. Evaluation and regression testing

Test	Expected behavior
Directly supported question	Answer is supported by retrieved text and cites the correct source and page.
Question requiring two source sections	Combines evidence carefully and cites each relevant source.
Unsupported question	Abstains rather than answering from model memory.
Partially supported question	Answers only the supported portion and states what remains unknown.
Wrong-audience retrieval	Does not apply adolescent, perinatal or workplace guidance indiscriminately.
Legal or medication question	Does not invent legal conclusions or medication instructions.
Crisis-related message	Safety pathway runs even if retrieval fails.
PDF added or updated	Index reflects the new version without duplicate or stale chunks.
Citation integrity	Every citation resolves to a real indexed source and page.

Track retrieval relevance, evidence faithfulness, citation correctness, abstention quality, response latency and safety-test performance. Maintain a hand-checked set of supported, unsupported and high-risk test questions and rerun it after changes to chunking, embeddings, prompts or models.

16. Source map for common user questions

Question category	First sources to retrieve	Boundary
Everyday stress and grounding	WHO, Doing What Matters in Times of Stress; WHO psychological self-help manual	Use original instructions for specific exercises; no guaranteed outcomes.
Structured self-help	WHO Psychological Self-Help Interventions (2026)	Do not infer protocols or eligibility criteria not present in retrieved text.
Student and adolescent well-being	WHO adolescent promotive/preventive guidance; WHO/UNICEF child and young people service guidance	Use age-appropriate language; do not diagnose or treat children independently.
Perinatal concerns	WHO guide for integration of perinatal mental health in maternal and child health services	Encourage qualified care; do not provide individualized medical or medication instructions.
Workload, stress, managers and workers	WHO guidelines on mental health at work; WHO/ILO policy brief; ILO psychosocial working environment report	Separate organizational risk management from individual coping; no unsupported legal advice.
Returning to work	WHO guidelines on mental health at work	Retrieve the original recommendations and preserve context.
Employment and participation	WHO guidelines on mental health at work; WHO/ILO policy brief	Avoid assumptions about capability or eligibility.
Question outside the corpus	No suitable source found	Abstain or ask a clarifying question; do not use external or pretrained knowledge if document-only mode is required.

17. Maintenance and responsible use

- Review the document collection periodically for new editions, corrected PDFs and extraction failures.
- Keep a record of source title, publisher, year, file checksum and ingestion date.
- Review the quality and provenance of stress Management.pdf before treating it as an authoritative source.
- Respect each document's licence, especially for redistribution, commercial use or derivative materials.
- Do not use the WHO, UNICEF or ILO logos or imply that these organizations endorse the chatbot.
- Keep private user messages and journal entries out of the shared knowledge base unless a separate, consent-based design explicitly requires it.
- Make privacy, retention and deletion behavior clear to users if the application stores chat or mood information.

18. Practical chatbot examples

User: “I have three exams and I feel overwhelmed.”

Response approach: Acknowledge the pressure, ask which exam or deadline is nearest if needed, and help select one small task. Retrieve the WHO stress guide for grounding or values-based action if relevant. Avoid promising a result or prescribing a rigid plan.

User: “My workplace is making me stressed.”

Response approach: Ask whether the main issue is workload, uncertainty, conflict or another factor. Use WHO/ILO workplace sources for general information on psychosocial risks and organizational responsibility. Do not imply that individual coping is the only solution.

User: “My teenager is withdrawn and upset.”

Response approach: Encourage calm, non-judgmental listening and appropriate safe adult or professional support. Retrieve WHO/UNICEF guidance relevant to young people. Do not diagnose the teenager or assume the cause.

User: “I feel distressed after childbirth.”

Response approach: Respond sensitively, avoid blame, and encourage contact with a qualified health professional or maternal and child health service. Use the original perinatal guide for source-grounded information. Do not diagnose or recommend medication.

User: “Tell me a fact not covered in my PDFs.”

Response approach: If strict document-only mode is enabled, explain that the approved documents do not provide enough information. Do not silently supplement with general model knowledge.

Appendix A. Uploaded source bibliography

- World Health Organization. *Doing what matters in times of stress: an illustrated guide.* Geneva: WHO; 2020.
- World Health Organization. *Psychological self-help interventions: delivering self-help for individuals, featuring Step-by-Step and Doing What Matters in Times of Stress.* Geneva: WHO; 2026. Generic field-trial version 1.0.
- World Health Organization. *WHO guidelines on mental health at work.* Geneva: WHO; 2022.
- International Labour Organization. *The psychosocial working environment: global developments and pathways for action.* Geneva: ILO; 2026.
- World Health Organization and International Labour Organization. *Mental health at work: policy brief.* 2022.
- World Health Organization and United Nations Children's Fund. *Mental health of children and young people: service guidance.* Geneva; 2024.
- World Health Organization. *Guidelines on mental health promotive and preventive interventions for adolescents: helping adolescents thrive.* Geneva: WHO; 2020.
- World Health Organization. *Guide for integration of perinatal mental health in maternal and child health services.* Geneva: WHO; 2022.
- Uploaded file: *stress Management.pdf*. Confirm the author, publisher, date and content scope from the PDF itself before treating it as authoritative.

Appendix B. Final implementation checklist

- Original PDFs remain available and are indexed alongside this synthesis.
- Every indexed chunk preserves original filename, title, page and section metadata.
- Answers are based only on retrieved passages in document-only mode.
- Unsupported questions lead to abstention rather than guessing.
- Every citation is validated against retrieved chunks.
- Children, adolescents, perinatal users and workplace users receive context-appropriate answers.
- Diagnosis, medication instructions and unsupported legal conclusions are not provided.
- Crisis handling is separate from normal retrieval and uses verified current contacts.
- Tests cover supported, unsupported, partially supported and safety-critical questions.
- Privacy, retention, deletion and source-licensing practices have been reviewed.

Closing note

A strong mental-health RAG assistant is not created by document length alone. It depends on a relevant source collection, careful extraction, well-designed chunks, accurate metadata, evidence-aware generation, reliable abstention, safety handling and ongoing testing. This detailed guide is intended to improve the structure and breadth of your knowledge base while preserving the original PDFs as the primary sources.